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OPERATIONAL POLICIES: FACILITIES, STAFFING, LOGISTICS & STANDARDS OF CARE

Peripheral Nerve Blockade: Stop Before You Block (Prep-Stop-Block)

YG OPG 016

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Key Points

- Wrong Side Blocks (WSBs) are a "Never Event".
- "Stop before you block" (SBYB) should be performed using a 3 step process:- "Prep-Stop-Block".
  - Step 1 Prep – Sign in / Drugs and equipment / Positioning / Preparing site/Gloving
  - Step 2 Stop – 2 person step, immediately before needling, site confirmed with surgical mark/consent/patient (if not sedated/anaesthetised)
  - Step 3 Block – Assistant hands tray to Blocker, Blocker immediately performs block

Background

Inadvertent WSBs are uncommon but can cause nerve injury, local anaesthetic toxicity, delayed hospital discharge, patient anxiety and distress and even lead to the risk of wrong-side surgery. WSB is a Never Event.

The SOP describes when and how 'Stop Before You Block' should be performed under a common framework. The emphasis is on a 3-step process consisting of preparation and a stop moment *just* prior to blocking (Prep-Stop-Block) that ensures there is a stop moment *just* before the block.

Stop Before You Block Process

Stop Before You Block is a three-step process of undertaking any peripheral nerve block or local anaesthetic infiltration:

Step 1 = Prep; Step 2 = Stop; Step 3 = Block

Step 1: 'Prep'; Preparation

Sign-in; On the arrival of the patient, the WHO 'Sign In' is performed.

Drugs and Equipment; the Blocker draws up the local anaesthetic solution and places the labelled syringe, along with a suitable nerve block needle, in a dedicated tray/container, separate from intravenous drugs.
This tray/container is then handed over to the Assistant, out of the Blocker's immediate reach.

Positioning: The Blocker and Assistant should position the patient and equipment in the final position ready for the block. Where necessary for the conduct of the block, any dressing or plasters casts should be removed. A 'pre-scan' ultrasound might be undertaken at this stage.

Preparing site/gloving: The Blocker dons gloves and prepares the site (cleaning, draping) suitable for the block. Where drapes cover the surgical site mark, the Assistant should ensure they can later reveal the mark for the Stop moment. Ideally, the surgical site mark should always be visible.

Step 2: Stop moment

The stop moment is a two-person step that happens only after preparation is complete and thus *immediately before* needle insertion.

When the Blocker is ready the Blocker formally announces they have completed all preparation and are ready to block using a consistent form of words: "*I've completed my prep; let's Stop Before You Block*". The Assistant similarly should reply: *"OK, let's Stop Before You Block".*

The Blocker and Blocker's Assistant together should then check the block side by viewing the surgical site mark, verbally confirming the correct side: the Assistant reconciles this with the consent form. If the patient is awake and unsedated, they may also confirm the side is correct.

Step 3: Block

Only when the correct side is confirmed does the Assistant hand the tray/container to the Blocker. The Blocker *immediately* performs the block.

Any delay between handing back the tray and/or performing the block should require the Blocker and Assistant to re-start the SBYB process at Step 1. Re-start might be at the very start of the Preparation stage or later, depending on circumstance; but will always involve first *handing the tray back* to the Assistant.

Recording and Analysing the process

After the block, the Blocker and Assistant should record that the SBYB protocol was followed.

References

SOP For Safe Performance Of Peripheral Nerve Blockade: Stop Before You Block (SBYB). Available at http://www.ra-uk.org/index.php/stop-before-you-block

Stop Before You Block Campaign. RA-UK. Available at http://www.ra-uk.org/index.php/stop-before-you-block

Safe Anaesthesia Liaison Group, SALG at: www.salg.ac.uk